Stakeholders list, stakeholders map and empathizing strategy

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1 Stakeholders list

Figure 1: Stakeholders identified in the project

2 Stakeholders map

Based on the CLIP stakeholder's classification, we ranked the different stakeholders into categories.

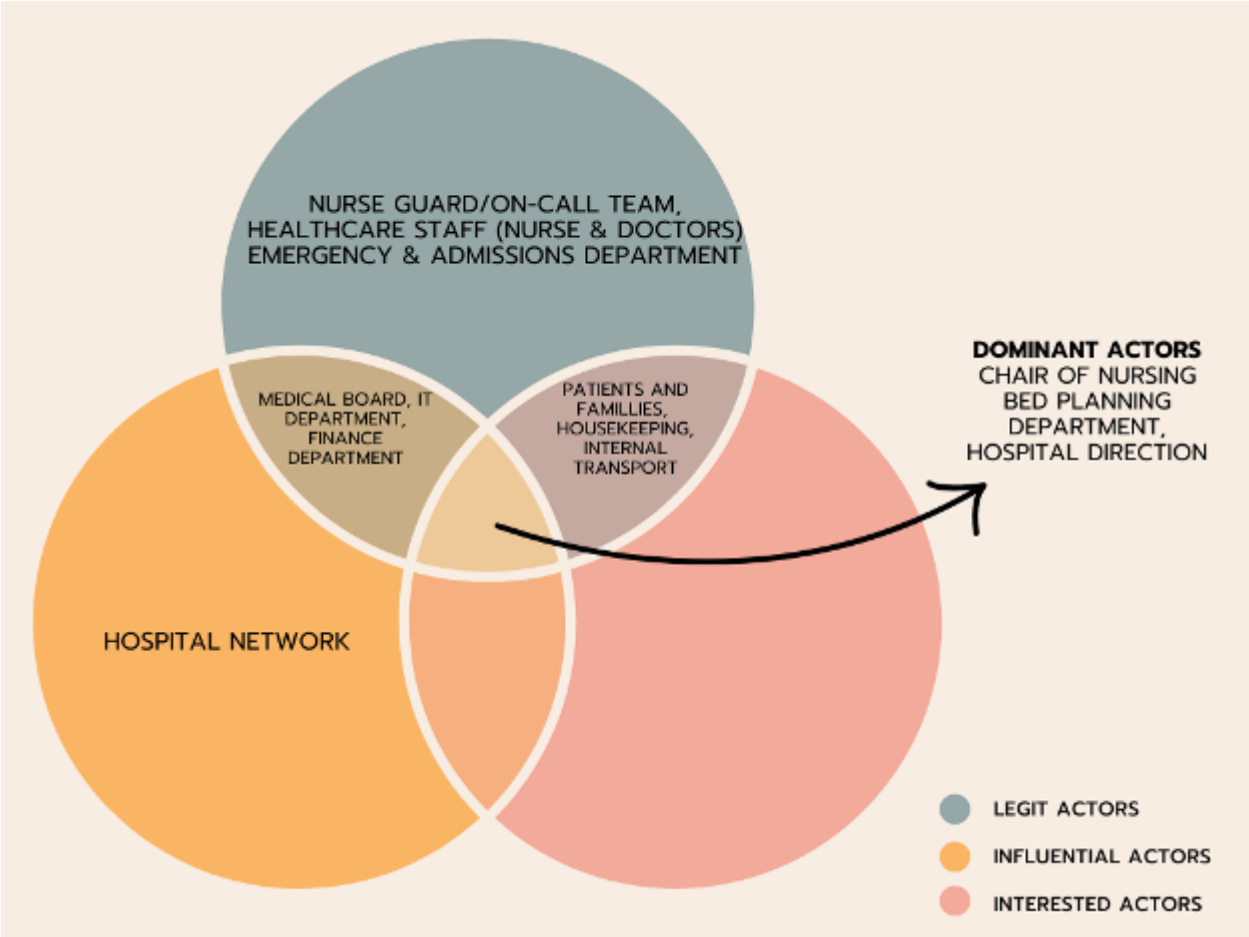

Figure 2: CLIP-based stakeholder map (legit, influential and interested actors)

Dominant Actors (High Power / High Interest)

- Chair of Nursing and Allied Services (Carol-Anne Jablonowski): Project leader with the formal authority to implement new nursing protocols.
- Bed Planning Department: They are at the heart of the daily struggle; they have the mandate to manage flow and a high interest in reducing "random occupancy."
- Hospital Direction: They provide strategic mandate and resources. They want the stable occupancy rate mentioned in the project goals.

Influential Actors (High Power / Low to Medium Interest)

- Medical Board: Since discharge is a medical decision, their agreement is mandatory for any new rule. They have high power but might have lower interest in "logistics" unless it improves their clinical work.
- IT Department: They hold technical power. Without their collaboration, the "optimization" (dashboards or tracking tools) cannot be realized.
- Finance Department: They have the power of the budget. They care about the "cost of unused beds," but they are not involved in the day-to-day nursing process.

Respected / Legit Actors (Medium Power / High Interest)

- Nurse Guard / On-call Team: They deal with the "refusals" from units every day. They have the experience to say what is realistic.
- Healthcare Staff (Nurses & Doctors): They are the ones who "justify refusals." Their interest is high because the current process is "time-consuming," but they might resist if they feel more pressure.
- Emergency & Admissions Departments: They are the "victims" of the current bottleneck. They have high legitimacy in demanding a faster bed turnaround.

Vulnerable / Passive Actors (Low Power / High Interest)

- Patients and Their Families: They suffer from delays in discharge papers or lack of rooms, but they are not part of the decision-making process.
- Housekeeping (Room Cleaning): They are often blamed for delays but usually just follow orders. They have a high interest in a more "predictable" cleaning schedule.
- Internal Transport / Porters: Their efficiency depends on a process they don't control.

Marginalized / Non-Actors (Low Power / Low Interest)

- Hospital Network (Réseau Namurois): While the clinic is part of this network, they are currently external to this specific internal process optimization, unless the project scales up to a regional level.

3 Empathizing Choices to Interview

Patients and their Families (Vulnerable)

They are the most impacted by delays but have no power; empathizing with them reveals the "human cost" of inefficient discharge (waiting in corridors, stress of transport).

Housekeeping / Room Cleaning (Marginal)

Often overlooked in strategic meetings, they are the physical bottleneck; understanding their constraints is key to reducing the time between a patient leaving and a bed becoming available.

Healthcare Staff: Nurses & Assistants (Respected)

They are the "legitimate" actors on the field; you must understand their reasons for "refusing" beds (fear of workload, lack of trust) to ensure they adopt your future solution.

Bed Planning Department (Dominant)

They hold the formal mandate to manage flow but suffer from poor data; empathy helps identify the "informal signals" they use to guess bed availability in a chaotic system.

Medical Board / Senior Doctors (Powerful)

They have the ultimate power to sign discharge papers; understanding their clinical priorities and why they might delay a discharge is essential to change the overall process rules.

4 Progress status

We secured an interview on Wednesday, 4 March with two important stakeholders:

- Ms. Felten (Planning Manager): She represents the Bed Planning Department.
  CLIP Category: Dominant.
  Why: She has the formal mandate and the data. She is the one struggling with "supply and demand" daily.
- Ms. Even (Deputy Director of Nursing): She represents the Nursing Management.
  CLIP Category: Powerful.
  Why: She has the authority to change nursing protocols and rules across all sectors. She is the link between Carol-Anne's strategy and the ward reality.

By interviewing Ms. Felten and Ms. Even (Powerful), we get the "system" perspective. Their positions allow us to understand the viewpoint of dominant and powerful stakeholders within the organization. To further strengthen our empathizing approach, we would also like to interview the staff, considered a respected stakeholder, as well as the housekeeping team, viewed as a more marginal stakeholder. These additional interviews would provide a "reality-based" perspective and enrich our overall understanding.

5 Empathy Strategy: Solving the assumption, time-consuming & refusals

The goal is to bridge the gap between official management data and the daily reality of the wards.

1. Top-Down Approach (Remote Interview - March 4th)

- Stakeholders: Ms. Felten (Dominant) and Ms. Even (Powerful).
- Objective: Define what "time-consuming" means at the management level. We want to identify administrative bottlenecks (e.g., constant phone negotiations, manual data entry).
- Key Focus: Understand why current planning feels "random" from their perspective and establish KPIs (e.g., relocation rates, bed turnaround time) to measure the problem.

2. Bottom-Up Approach (Planned On-Site Visit)

- Stakeholders: Nurses (Respected) and Housekeeping/Patients (Marginal/Vulnerable).
- Objective: Validate the "justifications for refusal." We need to see if units block beds due to real workload issues, lack of tools, or a lack of trust in the central system.
- Key Focus: Uncover the physical reasons for "random" occupancy (e.g., waiting for doctors' signatures, families arriving late, or cleaning delays).

By comparing the Dominant/Powerful view (the "System") with the Respected/Vulnerable view (the "Reality"), you will identify exactly where the process breaks down. This ensures your solution isn't just a top-down order, but a tool that actually saves time for the staff on the ground.

6 Mysteries to Solve (The Unknowns)

Information missing from the documents that we need to uncover during interviews.

- The "Notification" Mystery: How does the Planning Dept. actually find out a bed is free? Is there a real-time digital update, or do they rely on manual phone calls from nurses?
- The "Hidden Language" Mystery: Why do justifications for refusal vary by unit? Is there an "informal code" (e.g., claiming a bed is "broken" or "reserved") used by staff to control their own workload?
- The "Morning Gap" Mystery: Why is anticipation difficult? Does the delay come from the doctor's late rounds, the family's availability to pick up the patient, or the wait for transport/ambulance?
- What is the official trigger/message that confirms that a room is ready for a new patient, is it digital or does it go by telephonic confirmation?

7 Real Problems to Understand (The Pain Points)

Identifying exactly why the process is "time-consuming" and "random."

- The Negotiation Burden: How many phone calls does the Planning Dept. (Ms. Felten) make per day just to "beg" for a bed? Is the issue a lack of beds or a lack of visibility?
- Conflicting Interests: The Planning Dept. wants 100% occupancy (efficiency), while the Ward Nurses want a manageable patient-to-nurse ratio (safety). How is this tension resolved daily?
- The Turnover Bottleneck: Is "time-consuming" actually the gap between a patient leaving and the room being cleaned? Does Housekeeping know immediately when a patient has left?
- What are the criteria used to settle when conflicts arise between occupancy goals and actual unit capacity?
- Is there data available to identify occupancy patterns?

8 Hypotheses to Validate (The Assumptions)

At the end of the interview, not at the beginning. Theories we suspect are true but need to confirm on-site.

- Hypothesis 1 (Behavioral): "Units 'hide' beds or delay announcing departures to protect themselves from receiving difficult new patients or being overwhelmed."
- Hypothesis 2 (Technical): "The current software is too complex or disconnected from the field, leading staff to manage everything via phone/paper instead of the digital system."
- Hypothesis 3 (Structural): "Relocations happen because the hospital lacks a clear 'buffer' zone, forcing patients into the wrong sectors when discharge timing is missed."
- Hypothesis 4 (Communication): "Frontline staff don't see the financial impact of an empty bed; they see it as a much-needed break, creating a misalignment with Management goals."

9 Interview Questionnaire to Discover Information, Verify Real Problems, and Test Our Hypotheses

- What are the current methods being used?
- How is the planning of discharges and admissions organized?
- What impact do these problems have on quality of care and patient satisfaction?
- Which improvements would you consider the highest priority?
- Have you already tested any solutions to optimize bed management? If yes, which ones? If not, why not?